4 Psychiatric Coercion: Ethical Conditions and Empirical Data

4.1 Introduction

Modern society knows various forms of coercion. These range from the threat of fines, such as those related to compulsory seat belt use in cars or a smoking ban on aeroplanes, to the deployment of a special police task force to put an end to a hostage situation. Psychiatric coercion is a variant of general societal coercion. Since psychiatric coercion formally takes place in the context of medical care, it requires special legitimisation, as described in the previous chapter. One of the most important ethical and legal arguments for the justification of coercion in medicine in general and in psychiatry in particular is that the measure must ultimately be for the benefit of the person concerned. The person concerned should, according to the aforementioned criterion of the German Ethics Council, realise in retrospect that the measure was justified and that they benefited from it insofar as the absence of the measure would have led to harm to health. This also implies the effectiveness of the entire treatment. In addition, the measure must be used as a last resort when all others have been exhausted, and it must be the least restrictive possible. Finally, it is assumed that the measure restores the person's autonomy. However, is this truly the case? What does empirical research say about ethical conditions? The following conditions for the use of psychiatric coercion are therefore considered below:

1. Coercion is for the benefit of the person concerned.
2. The least possible restriction is used as a last resort.
3. Psychiatric therapies are effective.
4. The coercive measure restores the autonomy of the person concerned.

Another fundamental condition is the presence of a mental illness. As this topic is very complex and multi-faceted, it is addressed in detail in the next chapter.

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4.2 Condition 1: Is Coercion for the Benefit of the Person Concerned?

Do people affected by coercive measures agree afterwards with what has happened? Does the stay or even the treatment against their will help them in terms of their health? And what about people who are unable to enter the primary labour market or get their own flat because they are denied this on the grounds of a mental disorder and supposedly for their own good? Do they ultimately benefit from this?

Before these questions can be answered in detail, it is first necessary to consider the perspective from which a person's welfare should be viewed. From an ethical and legal perspective, as has already been stated on various occasions, it is the perspective of the person concerned. However, in practice but also in research, this is not always the case. A systematic review of coercion in the context of diagnosed eating disorders, for example, concluded that the measures are justified, as people affected by coercion would improve their body mass index just as much as those treated voluntarily [1]. This view is also paradigmatic for forensic psychiatry. There, among other things, the risk of criminal recidivism is seen as a main parameter of treatment outcomes [2]. Although this is presumably also the view of some of the people affected, there are considerable doubts as to whether this is the case for the majority. The clinical view, which does not necessarily take into account the perspective of the people affected, continues to dominate in practice but also, to some extent, in law.

However, let us stick to the clinical perspective first. What are the effects of involuntary measures? Various reviews have analysed the short-term effects of chemical restraints. They unanimously conclude that these measures work as intended and are able to actually calm affected—usually aggressive or agitated—people [3–5]. The side-effect profile and safety of the measures are also rated as positive [6].

But what about the longer-term effects? Interestingly, the relevant review literature is much less positive. With regard to the clinical effects of involuntary hospitalisation, a recently published scoping review concluded: "The clinical outcomes of IPA (Involuntary Psychiatric Admission; DR) are a highly conflicting and debated topic" ([7]: 291). On the one hand, minor psychopathological symptoms are emphasised, while on the other hand, depressive, post-traumatic and suicidal reactions are highlighted. A somewhat older review on restraint came to the conclusion that "... restraint is still widely used in psychiatry wards (3.8–20%) even though its efficacy has not been demonstrated..." ([8]: 19). A lack of effectiveness is also attested to for coercive outpatient measures, which are known as community treatment orders (CTOs). These are particularly ineffective with respect to readmissions to inpatient treatment and other parameters of the utilisation of psychiatric services [9, 10].

The reviews just cited have a crucial methodological problem in that they cannot control that the same characteristics of the people in the intervention group and in the control group are actually present in the conditions to be compared. Strictly speaking, this is only possible in the case of randomised clinical trials (RCTs). However, random allocation to a group that is subject to coercion and one for which

this is not the case is impossible, not least for ethical reasons. One way out of this problem is the so-called emulation study, in which an RCT is copied to a certain extent by means of an observational study [11].

In this way, it is possible to come relatively close to the "gold-standard" RCT without performing randomisation. This emulation procedure was applied to two datasets, one from a hospital and one from a national dataset from Switzerland. The primary outcome indicator was the clinical assessment by practitioners on the Health of the Nation Outcome Scale (HoNOS). Both in the hospital [12] and in the national database [13], patients who had to undergo coercive measures had significantly worse HoNOS scores. In other words, even from an exclusively professional clinical perspective, patients with a coercive measure deteriorate significantly.

Now to the perspective of those affected by coercion. Numerous empirical studies have focused primarily on the effects of direct coercion, and there are now various systematic literature reviews on this issue. Among these systematic reviews, one review has explicitly focused on ethical problems and their consequences. As already mentioned in connection with the German Ethics Council, the welfare of the person concerned is regarded by the ethical literature as "...the most important justification for coercion" ([14]: 99). For example, antipsychotic medication against the person's will is considered justified because it is assumed to be better than no medication at all. However, as this review concludes, "although beneficence may be the justification, this may not ultimately be the case" ([14]: 100). This ethically motivated study also emphasises the extent to which coercion adversely affects the relationship between professionals and those affected and how much mistrust arises on the part of those affected.

A qualitative meta-synthesis has summarised the experiences of people who were initially hospitalised against their will and then had to undergo further coercive measures such as seclusion/isolation or restraint. The analysis yields the following conclusions, which are quoted extensively owing to their relevance:

> Patients in several studies believed that their involuntary admission had kept them safe at a time when they could not recognise the severity of their illness, but negative experiences were commonly described. (...) Additionally, this review found physical interventions, such as restraint and seclusion, were experienced particularly negatively by many patients and played an important role in the negative experiences reported in the majority of studies. This review also highlights the lasting impact of detention with a number of patients across studies reporting feelings of shame and marginalisation, particularly patients in forensic settings who had committed an offence as well. ([15]: 7)

Another systematic review analysed the consequences of restraint and seclusion/isolation for the people affected [16]. As this review is, in my opinion, the most comprehensive analysis, the summary from the discussion of the article is also quoted here at greater length:

> The identified literature strongly suggests that seclusion and restraint have deleterious physical or psychological consequences. The incidence of PTSD after seclusion or restraint ranges from 25% to 47%, which is not negligible, especially in patients with past traumatic events. The main diagnoses associated with the use of seclusion or restraint in the selected

> articles are schizophrenic, schizoaffective, or bipolar, currently manic disorders. Subjective perception has high interindividual variability and can be positive, with feelings of safety, help, clinical improvement, or evaluation as necessary. However, seclusion and restraint are mostly associated with negative emotions, particularly feelings of punishment and distress. Conclusions on protective or therapeutic effects of seclusion and restraint are more difficult to draw. Our results provide little evidence for these outcomes, but further research is clearly necessary. (16]: 13; citations not reproduced here)

A systematic review of qualitative studies focused exclusively on the effects of seclusion. This review concludes the following: "Thematic synthesis of the data revealed emotionally powerful themes which suggest that seclusion is an exceptionally challenging experience for psychiatric inpatients. (...) The process of it is frightening for patients and leaves them in a vulnerable state with inadequate resources available to help them to cope with the distress" ([17]: 281).

Yet another review examined the details of the experiences of people affected by coercive measures [18]. This work comes to a similar conclusion to the study just cited. The majority of experiences associated with coercive measures are negative, regardless of the type of measure. The following experiences are described in detail: Anxiety and post-traumatic stress; powerlessness, neglect, mistrust and loneliness; punishment, abuse and pain; anger, rage and bitterness; depression, grief; humiliation and shame; and loss of freedom and coercion. However, positive experiences such as help and support, necessity, reassurance, time to reflect, safety, control and prevention of violence were also reported in a smaller number of the original studies. In this overall context, it should also be borne in mind that physical restraint not only has psychological consequences but also—albeit rarely—leads to physical harm and even death [19, 20]. Deaths are usually caused by cardiac arrest, pulmonary embolism or thromboembolism.

The predominantly negative experiences apply not only to acute psychiatry but also, in particular, to forensic institutions. The summarised results of a review are also quoted here in full:

> In the available literature, restrictive practices have been associated with harmful consequences for users, including feelings of anger, aggression and anxiety as well as experiencing trauma, disorientation and experiencing neglect and abuse. (...) (R)estrictive practices can become part of a 'vicious circle' in which the psychological instability and stress they create lead to more risk-taking behaviour, which in turn leads to restrictive practices. ([21]: 80)

This corresponds with another review of studies from the forensic setting. There, the experience of many of those affected is summarised as a culture of "repression" [22]. Against this background, it is understandable that more contemporary concepts such as personal recovery find it relatively difficult to gain a foothold in forensic settings [23]. The prevailing rules and norms, the duration of the placement and, finally, the attitudes of the staff, which often tend towards a conventional ideology of punishment, obviously stand in the way of the development of an individual perspective of hope.

Other reviews of individual aspects of the experience of coercion are largely along the same lines as the work just cited [4, 24–29]. The majority of people affected do not experience psychiatric coercion as benevolent. However, there is apparently a minority of people affected by coercion who view this positively, and there are apparently certain situations in which people have experienced placement as a form of security.

These results are related to direct and ultimately physical coercion. However, according to a corresponding overview, even informal or "soft" coercion also leads to negative experiences. Furthermore, it is not surprising if the feeling of inferiority in the sense of fewer rights and less freedom of choice than other people arises [30].

In the introductory chapter, I referred to the problem of social coercion, which occurs when people are denied certain life prospects through administrative decisions, i.e. when people are not supported in entering the primary labour market or when people are denied living in their own home with reference to their mental health problems.

Unfortunately, there are no studies on the effects of these forms of coercion. This is probably because the withholding of life prospects has not yet been categorised as coercion. Nevertheless, statements can be made about the consequences of people with psychosocial problems not being employed in the primary labour market and the consequences of living in residential care or group settings.

The idea that people with psychosocial problems are not as resilient and have to adapt slowly to difficulties dates back to the early days of psychiatric rehabilitation. These ideas are referred to as the "stepladder principle" [31] or the "linear continuum" [32]. This is based on the expectation that people with psychosocial problems—as in other areas of medical rehabilitation—become increasingly burdened to recover. This means that they should move up from the workshop via a social or supplementary employment company into the general labour market. Alternatively, it means moving from a residential care setting via an assisted living community into one's own home.

However, unlike in many other medical fields, this step ladder principle does not work. As empirical studies have shown, only 10–15% of people actually achieve their goals via the step ladder [33, 34]. In contrast, the success rate for the principle of "supported inclusion", i.e. direct access to the labour market and one's own home, is significantly better [34–38]. For many people who are unable to gain a foothold in the primary labour market, this means that they have to live at or below the poverty line. This is the case for almost half of the people with severe psychosocial problems in Switzerland [39]. Social exclusion through poverty may even have increased in recent decades, as a Belgian study recently documented [40]. For people who cannot live in their own home, these "objective" disadvantages are not visible. However, the preference against a residential or shared flat setting is so pronounced that in a research project in which I was involved, we had to virtually discontinue the study because hardly anyone was willing to voluntarily live in such a setting for a longer period of time [36]. Surveys of people with pronounced psychosocial problems have shown that more than 80% of respondents opt for their own home [41]. To summarise, this means that the well-being of the person

concerned is difficult to achieve in the case of social coercion. The disadvantages of social exclusion in terms of working and contravening the preference for housing are more than clear.

Is coercion associated with benefits for the people affected? The answer is both from the clinical perspective of professionals and from the perspective of those affected, predominantly no. Coercion is obviously only experienced as helpful by a minority of affected people. Evidence-based medicine is based on the view that a medical intervention is, on average, more successful than no intervention or an alternative treatment ("average treatment effect"). From this perspective, psychiatric coercion is not evidence-based, as the people affected—on average—do not benefit. Therefore, the discussion about psychiatric coercion should actually be over at this point. As the benefits cannot be proven, coercion in psychiatry should no longer be legitimised.

4.3 Condition 2: Is the Least Possible Restriction Used, and Is It Used as a Last Resort?

Coercive measures in psychiatry should be avoided wherever possible. If they are used, other options must have been considered and rejected. However, is this truly the case? To answer these questions, we can only partially draw on systematic reviews. It is first necessary to look at what resources are available as alternatives in psychiatric care.

Coercive measures usually arise either as a reaction to an escalating situation outside the inpatient setting or as a reaction to a—usually—aggressive conflict in the hospital. Let us first look at the situations in hospitals. In most psychiatric institutions in the Western world, measures have now been established in which staff can learn organisational and personal strategies to reduce coercive measures. These measures range from de-escalation training to improved and individualised therapy and care planning. Systematic reviews have shown that these measures tend to have a positive effect. An umbrella review summarising several meta-analyses revealed that employee training alone is able to minimise restraints [42]. However, if training alone is sufficient to achieve this effect, then this is an empirical indication that restraints are apparently not always used as a last resort, as more and better training is able to reduce coercion.

Further confirmation of this fact came from a large randomised implementation study of guidelines for reducing coercion in Germany. This showed a reduction in coercive measures in both the intervention group and the control group [43]. This circumstance, known in empirical research as the "Hawthorne effect", can be interpreted to mean that even in the control group, taking part in a study apparently had a positive impact on the use of coercion. In other words, there would have been room for less coercion even before the study.

However, coercion reduction initiatives often cannot achieve what was intended [44]. This also applies to the currently best-known programme *Safewards*. Safewards aims to reduce conflicts in inpatient facilities and thus indirectly reduce restrictions

and coercive measures. Two recently published reviews reported a tendency to reduce these measures, but this was not the case in all the studies included [45, 46]. This means that it is possible to use certain alternatives before interventions such as restraint or seclusion/isolation are used.

However, the question is how well this works in everyday practice. We know that studies are one thing; routine practice is another. So-called wash-out effects often occur. This means that the effect disappears over time after the intervention has been introduced, with a certain amount of effort and attention. Reviews [47] and individual studies [48, 49] suggest that there is indeed still considerable room for improvement here. Many of those affected primarily perceive the attempt to prevent formal coercive measures via medication and indirect coercion, such as threats of consequences. However, from the point of view of those affected, this is not particularly helpful. Instead, the focus is on dialogue and asking about current needs. In contrast, employees are generally convinced that they have used the last resort [50]. However, they do not see these means as exclusively related to the persons directly affected. In their opinion, coercive measures are also used to restore security and order in the respective department [51, 52].

Another way of applying fewer restrictions, which has not been used sufficiently to date, is to open closed wards. Closed ward doors are intended to prevent people from absconding from the ward or hospital and then possibly committing acts that are dangerous to themselves or others. However, it has been known for decades how much closed doors lead to aggressive conflicts in psychiatric wards [53]. There are no high-quality reviews on the subject of door openings, but both long-term retrospective observational studies [54] and prospective quasi-experimental studies [55] are available. These studies suggest that opening the door is not associated with an increased risk of suicide or other adverse events. A Norwegian study explicitly tested this non-inferiority with a corresponding study design and was able to impressively confirm the non-inferiority of open doors [56].

From the point of view of reducing coercion in hospitals, much would therefore be gained if the risk of hospitalisation could be reduced in advance. The intervention of choice is so-called "home treatment", which is described in German social law as "ward-equivalent treatment" and is known in British care as the "Crisis Resolution Team" [57]. Home treatment has been introduced in many countries, albeit with major differences and varying degrees of success [58]. Most home treatment programmes, such as those in Switzerland [59] and the Netherlands [60], succeed in reducing inpatient treatment days but not admissions. This is presumably due to the exclusion criteria for treatment in the home environment, which provide for inpatient admission if a certain risk threshold is exceeded.

In addition, however, home treatment programmes are not yet widespread outside the British Isles. In Switzerland, they only exist in a few locations, and even in Germany, where "ward-equivalent treatment" has been announced with great vigour, the number of cases is a single-digit percentage of all treatment cases [61].

In many cases, home treatment may also take place too late to prevent hospitalisation. The escalation may often already be too far advanced. A comprehensive study of the risk factors for involuntary admission in Switzerland, involving several

thousand cases, revealed how relevant the factors of aggressiveness and on the part of those affected contribute to the risk of admission [62]. Various systematic reviews have shown the relevance of so-called advance directives in this context [63, 64]. These are measures that create trust that the support system will fulfil the wishes of the affected person, even in the context of a serious psychosocial crisis. According to the reviews cited, these measures are actually the only ones that contribute significantly to less coercion.

A residential support programme that takes place in advance could also help prevent crises from developing by building trust in the support system and providing support with everyday problems. Indeed, as reviews suggest, such programmes appear to do this [65, 66]. In our recent supported housing study in Zurich and Bern from 2020 to 2022, we explicitly examined this phenomenon and were able to clearly confirm a reduction [36]. In another study with a different sample, we were also able to show that residential coaching (as we call it in Bern) significantly reduced the risk of inpatient admission [67].

Ultimately, the aim is to extend the continuum of approaches to crisis intervention so that the appropriate response can be given to people seeking help and that they are not deterred by the threat of coercive measures when they are looking for support [68]. This also includes services such as peer respites [69] or crisis cafés [70], which can offer low-threshold counselling and support and dispense with immediate clinical measures, which might then automatically lead to psychiatric coercion.

To summarise, there are, in principle, measures available both in inpatient and outpatient settings, but they are not currently used consistently enough or across the board. Despite all the efforts on the part of professionals, it cannot be concluded that the principles of last resort and least possible restriction are actually followed in every case.

4.4 Condition 3: Is Psychiatric Treatment Effective?

Coercive measures in the psychiatric support system have the ultimate goal of contributing to the health of the people affected by the measures. It is therefore not about the defence against a risk or harm but about the therapeutic effectiveness of the entire treatment associated with the measure. However, how effective is psychiatric treatment? Admittedly, this is not the first time this question has been asked in connection with coercive measures. As early as 1987, Mary Durham and John La Fond addressed the following analytical questions in the context of the US legal system and the US federal government's efforts at the time to restrict the rights of people with mental health problems after years of deinstitutionalisation: "Does it in fact help the individuals whom the government is purportedly seeking to help? Is there any evidence to establish that it does? Is there evidence that such persons might in fact be harmed rather than helped?(...) [I]t is crucial that available empirical evidence be carefully scrutinized to see if the government can in fact carry its strong burden of demonstrating that it is able to accomplish the therapeutic purpose

for which it seeks to take away the liberty of a vast number of Americans" ([71]: 309f). The answers the researchers gave were sobering. Instead of a therapeutic benefit, the findings at the time were more of a harm to people affected by coercion, for example, owing to the considerable side and long-term effects of the drugs.

However, what is the situation today and in view of the considerable changes in psychiatric therapy? A recently published umbrella review has provided an answer to this question [72]. In this type of review, a statistical methodology is applied to published systematic meta-analyses. The study therefore does not include individual studies, as in a normal meta-analysis but rather includes aggregated data from the reviews. Such umbrella reviews are particularly useful when many meta-analyses have already been published in a field of research such as psychiatric therapy research.

The aforementioned study included studies that covered 3700 randomised controlled studies with over 650,000 participants from noncoercive settings; the study therefore has considerable significance because of the number of studies included. The umbrella review then looked at how effective both pharmacological and psychotherapeutic interventions were in relation to standardised psychopathological scales. The decisive indicator is the standardised mean difference, which in turn corresponds to the effect size of an intervention (Cohen's d; see [73]). This indicates how large the measured differences between the intervention groups and the control groups were. The intervention groups received the drug or therapy that was tested, and the control groups received the placebo drug or usual care (TAU, *treatment as usual*).

The standardised mean difference, i.e. the overall effect size of the pharmacological interventions, was 0.34 for psychotherapy and 0.36 for pharmacotherapy. An effect size of 0.5 or more is usually considered to be moderately effective, and 0.8 or more is considered to be highly effective. This means that psychiatric therapy, whether pharmacotherapy or psychotherapy, is not even moderately effective. Only a minority of people who are treated actually benefit from the treatment.

The size of this minority in pharmacotherapy with antidepressants was recently calculated as part of a meta-analysis [74]. This analysis was based on data from the US *Food and Drug Administration* (FDA). The advantage of this database is that no publication bias is expected in the data to be analysed. In meta-analyses of only published studies, there is the phenomenon particularly in pharmacological research, when studies without success for the intervention are sometimes not published and therefore cannot be included in meta-studies. On the basis of FDA data on antidepressant monotherapy, an effect beyond the placebo effect was shown in only 15% of those treated. Not to be misunderstood: this is not nothing, and I would also try an antidepressant myself first in the event of a depressive development. However, I have serious doubts as to whether this rather small effect can be used as justification for an effective psychiatric intervention, which in turn can be used to justify a possible coercive measure.

In the context of coercive measures, it should also be noted that the study participants are exclusively volunteers, who can be assumed to have a certain motivation to be treated and to change. In the case of involuntary treatment, this motivation, if

it exists at all, is significantly lower; therefore, an even lower level of effectiveness must be assumed here.

Another effect that leads to an overestimation of the reported effectiveness of psychopharmacological interventions is the empirical finding that participants in RCTs show better effectiveness than do those in so-called real-world studies [75]. The reason for this lies in the relatively strict inclusion criteria of RCT studies, which, among other things, mean that people with psychosocial problems who have multiple diagnoses (comorbidities) are generally excluded. The exclusion is intended to relate the measured effectiveness to the particular psychological problem and to avoid confounding factors.

Suicidality is the most common indication for coercive psychiatric measures, alongside risk and aggression to others. It is therefore worth examining the effectiveness of therapeutic interventions with respect to suicidality and completed suicides. A comprehensive meta-analysis with RCTs from 50 years with all possible interventions revealed an effect size that cannot even be interpreted as small (Hedge's $g = 0.17$) [76]. As with the effect size Cohen's d, from 0.2 onwards, we speak of a small effect, and from 0.5 onwards, we speak of a moderate effect. Two things were particularly interesting about this study in addition to the central result: first, no intervention was identified that was significantly more effective in the treatment of suicidality than others were, and, second, there was virtually no evidence about the effectiveness of acute psychiatric treatment, as is usually associated with compulsory hospitalisation due to suicidality.

Another meta-analysis focused on pharmacological and somatic interventions such as electroconvulsive therapy [77]. This analysis revealed significant effects on completed suicides only for lithium for people diagnosed with bipolar disorder and clozapine for people diagnosed with psychotic disorder. All other antipsychotics and electroconvulsive therapy showed no significant results. Yet another meta-analysis focused on pharmacological interventions. This showed a low effect size of 0.21 across all the medication classes [78]. However, researchers have emphasised the possible negative effects of certain medications, which can even increase the risk of suicide. A corresponding meta-analysis reported this for the selective serotonin reuptake inhibitor (SSRI) class of antidepressants [79].

For the sake of completeness, it should be emphasised that psychotherapeutic procedures also have only minor effects on suicidality, as a recent review has shown [80]. The same applies to brief aftercare contacts following discharge from inpatient treatment. These transitional interventions were able to influence suicidality to a small extent but did not affect the likelihood of completing suicide [81].

I already noted at the beginning of this chapter that criteria are applied in the context of compulsory admission and compulsory treatment, which are not necessarily relevant from the perspective of the person concerned. Corresponding meta-analyses from forensic settings report, among other things, positive developments in criminal recidivism. However, they also reported a comparatively high suicide mortality rate among people who were released from forensic treatment [82]. In line with the umbrella study described above, psychotherapeutic and pharmacotherapeutic reviews from forensic institutions have also emerged with only weak effects

[83] or had to concede that the original studies were not very meaningful [84]. A similar situation can be observed when reviews of psychotherapeutic and pharmacotherapeutic interventions for individuals diagnosed with disorders, such as dissociality or sexual offences, are examined. There is hardly any evidence of effective procedures for either [85–87].

In addition to forensic institutions, various individual studies have been conducted on the outcomes of involuntary treatment, but I was only able to identify two systematic reviews. One of these is not entirely new [88], and the other summarises only studies from Norway [89]. Both studies contain a large number of studies with heterogeneous outcome indicators, which, unfortunately, are not particularly informative for the research question.

The legal question cited at the beginning regarding the benefits of psychiatric therapy in the context of coercive measures must therefore continue to be answered as rather questionable. Against the background of the evidence referred to in condition 1, a benefit that is to be assessed as low must be set against an expected harm for the persons affected by coercion.

4.5 Condition 4: Can the Autonomy of the Persons Concerned Be Restored?

A central generalised assumption when deciding on coercive measures is the reduced autonomy of the person concerned. It is believed that "mental illness" impairs the ability to form self-determined and free will. The person's decisions against treatment are therefore overridden by the illness. Conversely, the treatment associated with the measure aims to restore autonomy, which is postulated accordingly in the legal literature [90, 91]. However, is this even possible? Is autonomy diminished by the illness, and can it be restored?

To answer these questions, we are only partially involved in empirical research. The problems of autonomy and free will are mainly discussed in philosophy. The problem for the question at hand here, however, is that these terms—as is often the case in philosophy—have very different meanings [92]. However, this applies not only to theoretical analyses but also to everyday life in psychiatric institutions. While from a medical perspective, a person's autonomy is impaired by the illness [93], affected persons are more likely to experience that their autonomy is reduced by coercive measures [94].

What is the significance of autonomy and free will in the context of psychiatry? Reduced culpability in connection with a psychosocial problem is an issue that is particularly familiar with legal proceedings. This issue, known as the *insanity defence*, is particularly relevant in criminal proceedings when defendants are not sentenced to the usual punishment but instead receive other forms of punishment owing to their mental state, such as therapy conditions or placement in a forensic setting. Ultimately, diminished culpability is used as a construct to avoid injustice and to protect people with pronounced psychosocial problems from a prison sentence [95].

From a human rights perspective, the distinction between people who have committed offences under the influence of a mental health problem and those for whom this was not the case is certainly problematic. There is even a view that sees this as discrimination and demands equal treatment [96]. In this respect, the question of whether actions are fundamentally autonomous and conditioned by free will or whether this is not the case is of fundamental importance.

In philosophy, certain perspectives involve autonomy and free will in a relevant context. Autonomous decisions are, according to one view, determined by free will [97]. However, as might be expected, this view is not uncontroversial [98]. A fundamental discussion revolves around the question of whether actions and decisions are biologically determined. Since every sequence in a cascade to an action is preceded by other steps, the debate centres on how much freedom can actually exist. In addition to this determinism/indeterminism debate, there is another debate about the extent to which a deterministic view does not enable free will. It is therefore about the compatibility of determinism and free will.

From a purely scientific point of view, indeterminism and the compatibility of determinism and free will are not fundamentally excluded [99], but they are hardly plausible. The biologist Robert Sapolsky once dryly remarked that free will is what biology has not yet discovered [100]. From an overall perspective, I do not necessarily adopt this perspective if I find it plausible. To summarise the question at hand, this means that from a philosophical perspective and from a neuroscientific perspective, it is at least unclear whether humans have something like autonomy and free will at all. In fact, in my personal opinion, there is a great deal to suggest that this is not the case. However, if it is at least unclear that people even have a form of autonomy that can be impaired by a mental disorder, then there are also very serious doubts as to whether psychiatric therapy can restore this autonomy through coercive measures.

Elsewhere, the previously cited biologist Robert Sapolsky expressed the hope "... that when it comes to dealing with humans whose behaviors are among our worst and most damaging, words like 'evil' and 'soul' will be as irrelevant as when considering a car with faulty brakes..." ([101]: 611). If the mechanics experts cannot find a source of the fault and the defect persists, we will not be talking about a "bad car" either, Sapolsky continues.

If I think this example through further, this means that just as the question of further use arises with cars, the question must be asked with people with dangerous tendencies as to how they can be dealt with in a humane way so that they pose as little danger as possible to their fellow human beings. I attempt to answer this question in Chap. 8. It should already have become clear here that a relatively deterministic view of the human psyche does not necessarily have to result in dehumanisation. According to Sapolsky, such a way of thinking "... is a hell of a lot more humane than demonizing and sermonizing them as sinners" ([101]: 612).

4.6 Conclusion: Ethical Assumptions and Empirical Data

The above remarks should clarify how much the ethical assumptions and justifications differ from the results of the relevant empirical research. Psychiatric coercion is predominantly not carried out for the benefit of the person concerned. Additionally, measures against the will are generally not used as a last resort and as the least possible restriction. There are usually more options than are implemented in the respective care systems. There is considerable doubt about the effectiveness of psychiatric therapies in general, especially in the context of involuntary settings. There are at least many doubts regarding the assumption that a person's autonomy can be restored by means of coercive measures. Another important prerequisite for the use of coercion in psychiatric settings, namely, the definition of a mental disorder, remains to be clarified. This is done in the following chapter.

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